

1. Administrative & Study Identification

Full Study Title: A Phase II, Multicenter, Open-label Study to Evaluate the Efficacy and Safety of Trastuzumab Deruxtecan (T-DXd, DS-8201a) as Monotherapy or in Combination With Anti-cancer Agents for the Treatment of Patients With Selected HER2-expressing Tumors (DESTINY PanTumor03) **Short Title/Acronym:** DESTINY-PanTumor03 **Protocol Number:** D781AC00001 **NCT Number:** NCT06271837 **Posting ID:** 1142284136122137392 **Trial Status:** RECRUITING **Sponsor/Company Name:** AstraZeneca **Investigational Product(s):** Trastuzumab deruxtecan (T-DXd / DS-8201a / Enhertu) as monotherapy or in combination with anti-cancer agents, including Bevacizumab (Avastin, Vegzelma, Alymsys, Zirabev, Mvasi). **Phase of Development:** Phase II **Medical Monitor:** AstraZeneca Clinical Study Information Center / Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, specifically the confirmed Objective Response Rate (ORR), of trastuzumab deruxtecan (T-DXd) as monotherapy or in combination with anti-cancer agents for the treatment of patients with locally advanced, unresectable, or metastatic selected HER2-expressing solid tumors. **Secondary Objectives:** To assess the Duration of Response (DoR), Disease Control Rate (DCR), Confirmed Best Objective Response (BOR), and safety/tolerability of the treatment.

2.2 Background & Rationale To address the critical unmet medical need in patients with Advanced Solid Tumors (Excluding Gastric Cancer and Breast Cancer) who are not eligible for curative therapy. HER2 overexpression is responsible for cell proliferation, differentiation, and survival, linking it to tumor aggressiveness and a weaker response to standard chemotherapy. T-DXd is a highly effective HER2-targeted antibody-drug conjugate (ADC) designed to deliver a potent cytotoxic payload directly to cancer cells, providing a targeted mechanism to improve clinical outcomes across pan-tumor indications.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Non-randomized, single group assignment) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Targeted Enrollment: 175 patients **Number of Sites:** Multicenter **Current Status:** Recruiting **Estimated Study Start:** 01-Aug-2024 (Q3 2024) **Estimated Primary Completion:** 31-Oct-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. At least 18 years of age. 2. Locally advanced, unresectable, or metastatic solid tumors based on most recent imaging, with confirmed HER2 expression. 3. ECOG performance status of 0-1. 4. Must provide an adequate FFPE tumor sample to centrally determine HER2 status and other correlatives. 5. Has measurable target disease assessed by the investigator based on RECIST 1.1. 6. Adequate organ function and bone marrow within 14 days before enrollment. 7. Contraceptive use by males or females consistent with local regulations; evidence of post-menopausal status or negative serum pregnancy test for females of childbearing potential. 8. Capable of giving signed informed consent (ICF) prior to mandatory study-specific procedures.

4.2 Exclusion Criteria 1. Primary diagnosis of adenocarcinoma of the breast, adenocarcinoma of the gastric body, or gastroesophageal junction. 2. History of (non-infectious) interstitial lung disease (ILD)/pneumonitis that required steroids, has current ILD/pneumonitis, or suspected ILD/pneumonitis cannot be ruled out by imaging. 3. Has any spinal cord compression, leptomeningeal disease, or clinically active CNS metastases. 4. A pleural effusion, ascites, or pericardial effusion that requires drainage, peritoneal shunt, or cell-free and concentrated ascites reinfusion therapy. 5. History of another primary malignancy within 3 years before the first dose (with some exceptions for curatively treated low-risk tumors). 6. Unresolved toxicities from previous anti-cancer therapy, or anti-cancer chemotherapy without an adequate treatment washout period prior to enrollment. 7. Uncontrolled infection requiring IV injection of antibiotics, antivirals, or antifungals, active tuberculosis, active primary immunodeficiency, uncontrolled active HIV, or active Hepatitis B or C infection. 8. Protocol-defined inadequate cardiac function. 9. Major surgical procedure or significant traumatic injury within 4 weeks of the first dose, or an anticipated need for major surgery. 10. Known allergy or hypersensitivity to study treatment, any product excipients, or other mAbs. 11. Currently pregnant or breastfeeding, or planning to become pregnant. 12. Substance abuse or any concomitant medical condition that would increase toxicity risk or interfere with study participation/evaluation.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: T-DXd administered via intravenous (IV) infusion at the protocol-specified weight-

based dose on Day 1 of each 21-day treatment cycle, potentially in combination with anti-cancer agents (e.g., Bevacizumab). **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until objective disease progression, unacceptable toxicity, withdrawal of consent, or death.

5.2 Concomitant & Prohibited Therapy Permitted: Routine supportive care medications (e.g., antiemetics). **Prohibited:** Concurrent therapeutic clinical studies (excluding observational studies), concurrent anti-cancer therapy outside of the protocol, or systemic corticosteroids for conditions other than ILD management or standard premedication.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, FFPE sample collection, baseline tumor imaging, cardiac/lung screening
Baseline	Day 0	Cycle 1 Day 1: First Dose, PK sampling, Safety Labs
Interim	Every 3 Weeks (Q3W)	Safety Labs, IV Infusion, AE tracking. Tumor imaging every 6-8 weeks
End of Study	Variable	End of Treatment (EOT) Visit, Safety Follow-up, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Confirmed Objective Response Rate (ORR), defined as the proportion of patients who have a confirmed complete response or confirmed partial response. **Measurement Method:** Radiological imaging evaluated per RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Duration of Response (DoR) * Disease Control Rate (DCR) * Confirmed Best Objective Response (BOR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of hematologic toxicity, cardiac function (LVEF), and rigorous pulmonary imaging for Interstitial Lung Disease (ILD). **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee assigned to periodically review safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all efficacy evaluations; Safety Analysis Set for all patients receiving at least one dose of the study drug. **Sample Size Justification:** Targeted enrollment of $N=175$ is statistically powered to evaluate the robust ORR hypothesis across the pan-tumor cohorts. **Handling of Missing Data:** Standard survival analysis right-censoring rules applied for time-to-event endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote source data verification (SDV), with particular focus on RECIST assessments and ILD reporting. **Audit Trail:** Robust electronic case report form (eCRF) query resolution and audit trails maintained in the EDC system.

11. Study Identifiers & Governance

Primary ID: D781AC00001 **Registry Number:** NCT06271837 **Posting ID:** 1142284136122137392 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** DESTINY-PanTumor03 **Official Regulatory Title:** A Phase II, Multicenter, Open-label Study to Evaluate the Efficacy and Safety of Trastuzumab Deruxtecan (T-DXd, DS-8201a) as Monotherapy or in Combination With Anti-cancer Agents for the Treatment of Patients With Selected HER2-expressing Tumors (DESTINY PanTumor03)

12. Clinical Framework (Oncology/HER2 Specific)

Primary Condition: Advanced Solid Tumors (Excluding Gastric Cancer and Breast Cancer) **Diagnosis Threshold:** Confirmed HER2 expression **Medical Methodology:** HER2-targeted Antibody-Drug Conjugate (ADC) therapy alone or in combination with other anti-cancer agents (such as Bevacizumab) **Optimization Target:** Tumor regression (Confirmed Complete/Partial Response) and disease control

13. Study Procedures & Methodology

Management Pathway: Oncology treatment pathway for advanced, unresectable solid tumors **Education Protocol (HCP):** Mandatory training on ILD/pneumonitis identification, management, and toxicity monitoring **Education Protocol (Patient):** Strict guidance on symptom reporting (respiratory distress, cardiac signs) and

infusion schedule adherence **Quality Audit Mechanism:** Independent central review of RECIST 1.1 tumor assessments and continuous safety oversight

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression, unacceptable toxicity, or death **Onsite Visit Cadence:** Every 3 weeks (Q3W) for infusion cycles; tumor imaging every 6-8 weeks **Remote Monitoring**

Frequency: Intermittent monitoring between infusion cycles for AE tracking **Checklist Review Interval:** Survival follow-up every 12 weeks post-treatment discontinuation

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					